

Concept

We are directly going after the drug lag problem. Between 2005 and 2022, 78 drugs were approved in Japan but never in the United States. The broader addressable universe is larger: hundreds of drugs approved by FDA, EMA, PMDA have not been bridged into emerging markets where they are commercially viable. Each one is an already de-risked therapeutic with multi-year human data already treating patients on the markets with the highest standards.

The drug rights are sitting within US and European biotechs, NewCo platforms, and venture-backed development funds. An example is Catalys Pacific, their portfolio consists of ~10 drugs out of which not a single one is present within emerging markets. In their words “The juice is not worth the squeeze”. Each drug is conservatively worth \$5M+ per country per year in emerging-market (EM) revenue with modest commercial penetration. Our model is validated. Pint Pharma spent 30 years cultivating a portfolio of ~10 in-licensed drugs from across the globe. They had around \$60M annual revenue and it got acquired in September 2025 by Sobi (Nasdaq Stockholm: SOBI, ~\$15.9B market cap) for its LATAM regional distribution infrastructure.

We are building the AI-native, parallelized version. The wedge is Mexico: COFEPRIS launched an abbreviated reliance pathway in September 2025 granting 45-business-day reviews for drugs with a reference-authority approval (FDA, EMA, Health Canada, or PMDA) issued within the last 5 years, with onward cascade into 8+ regional markets from a single filing. We use AI agents to parallelize the full asset lifecycle, sourcing, bridge analysis, regulatory submissions, KOL mapping, patient population identification, payer access, across LATAM, MENA, and SEA simultaneously. We go boots-on-the ground and build the distribution networks and partnerships per country and aggressively in-license de-risked assets from top distributors.

The product we build and sell:

Sign 2-3 licensing deals with Catalys-like partners (~\$0-1M cash + NewCo equity + x.x% royalty per asset). Run the first asset bridges manually with embedded ops, instrumenting every step. **As patterns emerge we build agents to automate them**, pulling work back from external consultants, by month 24 we own much of the bridging process in-house. File COFEPRIS reliance pathway in parallel across 2-3 assets. First 1-2 Mexico approvals and commercial launches by end of Year 2, with cascade filings underway in either LATAM (Brazil/Colombia via PAHO reference) or the Middle East (Saudi SFDA verification). By month 18 we're in active talks with 3-5 new upstream partners (Fortress Biotech, BridgeBio, smaller specialty biotechs) on the next deal wave. Revenue: ~\$500K-2M per launched asset by end of Year 2 in Mexico.

The agent process scales across every emerging market with an FDA, PMDA, EMA, or Health Canada reliance pathway—most allowing clearance without local trials. Cascade Mexico approvals across LATAM via PAHO reference status (Brazil, Colombia, Argentina, Chile, Peru). Extend to MENA (Saudi SFDA Verification with FDA+EMA, UAE MOHAP, Egypt EDA, Jordan, Israel) and Southeast Asia (Singapore HSA Abridged as the regional gateway, then Malaysia NPRA, Thailand TFDA, Philippines FDA-PH, Vietnam DAV). Each new market filing runs in 3-6 months instead of 12-18 because the bridge dossiers and submission templates are already built — incremental cost per market drops 70-80%. Scale to 6-8 commercialized assets across 8-12 markets. By Year 4 deals come inbound: we're the lowest-friction emerging-markets path for any US/EU biotech holding unallocated rights, including post-FDA-approval rights from Catalys-style upstream partners whose Year 1-2 NewCos are now clearing FDA and need ex-US/EU commercialization. Revenue: 20M-70M ARR by Year 5.

Progress and secrets that informed the idea:

Source 1, a former Flagship Pioneering partner Hugo worked with, pulled back the curtain on the scale and growth of East-to-West drug licensing and the M&A opportunities in pre-clinical Chinese assets. We started EF planning to build an AI diligence assistant for China-to-US deals.

Source 2, the Chief Strategy Officer of a \$3.5B rare-disease biotech, who told us the AI diligence assistant play wasn't where the money was. Her own team had screened 100 assets in a year and didn't need another tool. She stated the unrealised opportunity was in already-approved Asian assets nobody had brought to the US, and she pointed us to her nephew, who was doing exactly that with a Japanese drug at a venture firm called Catalys Pacific. The thesis shifted: approved assets identification, not pre-clinical DD.

Source 3, a consultant at a boutique life sciences advisory, quantified it: 78 Japan-only approvals in 15 years that had not been out-licensed. He believed the arbitrage opportunity was real but stated Japan was culturally moated and difficult for smaller western biotech firms to access, it was his belief that big pharma wouldn't be interested).

If there was this arbitrage opportunity why had big pharma not exploited it?

Source 4, an executive at Pierre Fabre, told us many big pharma players don't want approved assets, since everyone internally is chasing the next Ozempic.

Source 5, the Head of Search and Evaluation, Asia at Novartis, told us they would in theory be interested in approved assets with a value over \$250M minimum, but global deals prioritized, first-in-class and innovative only. This was a key insight into why big pharma wasn't poaching these approved Japanese assets, many were high quality but not the next Ozempic / blockbuster and fell slightly under the \$250M threshold.

Source 6, an M&A associate on a healthcare-focused team, who gave us the pivot. Japan and Korea to US and Europe were already dominated by Catalys Pacific and AN Ventures, they had the firepower to fund +\$100m trials. He questioned the wisdom of fighting with them over the US and EU, but instead floated the question, Why not take the emerging market rights? Mexico's regulatory system allowed for near automatic approval for drugs that had been approved in Japan taking only 45 days.

Source 7, a Japan licensing expert, independently confirmed it on regulatory grounds: Japanese PMDA data has better acceptance in emerging markets than other Asian data. Brazil, Mexico, Colombia named explicitly. Furthermore, they confirmed that many biotech and big pharma players often own the global rights but don't execute the EM rights for years if at all.

Source 8, a former executive at EQRx, gave us the cautionary tale. EQRx had tried the China-to-US version of bridging assets (me-too drugs) and got killed when the FDA rejected Chinese-only data, destroying a company that had 1.8 billion in funding. Japan was structurally different: no data integrity concerns, GCP inspections work. With experience handling the Americas distribution for Merck, he handed us the LATAM operational playbook: bulk import, local CDMO packaging, local distributor partnerships, he said this was a better play albeit not the multi billion dollar US licensing play out the gate but one that had a pathway to scale and a real shot at working, especially when leveraging AI.

The updated thesis was now: approved Japanese assets, brought to LATAM then other Emerging Markets, via licensing with local execution partners.

Source 9, the Director of New Business Development at J&J Innovative Medicine APAC, stated that he had thought about this opportunity for several years, he said there were some players in the space but many had been acquired and slowed their LATAM licensing business. He said the key to operating this kind of business was less medical expertise but extremely thorough valuations assessments around who will pay for what drugs in what regions, then fast boots-on-the-ground execution operational excellence.

Source 10, the COO of Crystallys and a partner at Catalys Pacific, the industry leader of the Japan-to-US arbitrage (Since its inception in 2019, Catalys Pacific has been involved in every approved asset bridging from Japan to US), Catalys runs a 100-to-200-to-1 pass rate and has moved 12 drugs in 7 years, these approved drugs that have commercial potential and are likely to pass FDA trials are exceedingly hard to find. He told us US is 50% of their revenues, Europe another big chunk, but emerging markets, "the juice isn't worth the squeeze," and they haven't monetised any of their EM / LATAM rights. A \$50M LATAM forecast is a rounding error against a \$1B US opportunity. We asked would you partner or sell the rights "we'd be open to that" referring to in-licensing their emerging markets rights.

Why now:

The IRA + MFN are cutting US pharma revenue

US drug pricing reform. The Inflation Reduction Act (signed August 2022) gave Medicare the power to negotiate prices for high-cost drugs for the first time; ZS Consulting projects this will reduce US pharma company revenues by ~31% through 2039 and result in 135 fewer new drug approvals. Trump's May 2025 MFN executive order layered additional pressure: 17 major pharma manufacturers (including Pfizer, Merck, Novartis, BMS, Sanofi, GSK, Gilead, Amgen, AbbVie, Regeneron) have signed binding agreements committing to MFN pricing in Medicaid and to repatriate increased foreign revenue toward US discounts.

Emerging markets are outpacing US and EMA in growth

Frost & Sullivan forecasts YoY pharma growth in the US and Europe at low single digits versus stronger growth across APAC, LATAM, and other emerging markets. IQVIA projects LatAm pharma at a 22% CAGR through 2027 (partly inflation-driven via Argentina) and MENA pharma at ~\$57B in 2025 growing toward ~\$78B by 2033. BCG estimates ~\$275B of patent-cliff revenue at risk in Western markets between 2025-2030.

Reliance pathway regulatory inflection (last 12-24 months)

Mexico's COFEPRIS launched a 45-business-day abbreviated reliance pathway in September 2025 for drugs with FDA, EMA, PMDA, or Health Canada approval within the prior five years. Brazil's ANVISA finalized its reliance framework via RDC 913/2024; Saudi SFDA Verification and Singapore HSA Abridged are also operational reliance tracks.

AI capability

Bessemer Venture Partners estimates the outsourced pharma services market at \$150B+, with only ~5% of pharma companies having captured measurable AI value to date due to legacy data silos and seat-based vendor economics. The field is lagging, yet multiple regulatory-operations tasks (dossier translation, bridge analysis, deficiency-response drafting, multi-jurisdiction submission tracking) are now technically automatable with current LLM capability.

Glossary

Regulatory authorities

FDA — Food and Drug Administration (US)

EMA — European Medicines Agency

PMDA — Pharmaceuticals and Medical Devices Agency (Japan)

COFEPRIS — Comisión Federal para la Protección contra Riesgos Sanitarios (Mexico)

ANVISA — Agência Nacional de Vigilância Sanitária (Brazil)

RDC — Resolução da Diretoria Colegiada (Brazilian regulatory resolution)

SFDA — Saudi Food and Drug Authority

MOHAP — Ministry of Health and Prevention (UAE)

EDA — Egyptian Drug Authority

HSA — Health Sciences Authority (Singapore)

NPRA — National Pharmaceutical Regulatory Agency (Malaysia)

TFDA — Thai Food and Drug Administration

FDA-PH — Food and Drug Administration of the Philippines

DAV — Drug Administration of Vietnam

PAHO — Pan American Health Organization

Regions

US — United States

EU — European Union

LATAM — Latin America

MENA — Middle East and North Africa

SEA — Southeast Asia

APAC — Asia-Pacific

EM — Emerging Markets

Business and clinical terms

NewCo — New company (entity formed to hold and develop rights to a specific drug)

KOL — Key Opinion Leader (specialist physician who influences prescribing)

CDMO — Contract Development and Manufacturing Organization

GCP — Good Clinical Practice (international clinical trial standard)

DD — Due Diligence

M&A — Mergers and Acquisitions

CAGR — Compound Annual Growth Rate

YoY — Year-over-Year

ARR — Annual Recurring Revenue

LLM — Large Language Model

Policy

IRA — Inflation Reduction Act (US, August 2022)

MFN — Most Favored Nation (drug pricing policy)

Companies / tickers

SOBI — Swedish Orphan Biovitrum (Nasdaq Stockholm: SOBI)

BMS — Bristol Myers Squibb

GSK — GlaxoSmithKline

J&J — Johnson & Johnson

BCG — Boston Consulting Group